

CURAIS

Product Foundation

Brand story, product requirements, architecture, security, feature delivery,
and frontend direction.

Clear help now. Better care next.

The Curais Story

A name built around care

Curais begins with *cura* - the Latin root for care, healing, and attention. It is paired with AI, not to make technology the hero, but to make care more available when people need clarity.

Curais means: care, made more present by intelligence.

Why it matters

In a health moment, people do not need another complicated system. They need to know what to do now, who to call, and how to carry the right information into the next step of care. Curais starts there.

SnapAid becomes Curais Immediate Care: calm, actionable first-aid guidance at the moment of need. From that first moment, Curais can help people preserve the health context they choose to keep, share it safely with a clinician, and receive more continuous care over time.

Brand promise

Clear help now. Better care next.

Curais is designed to feel capable but never clinical or cold; urgent when a situation is dangerous, and calm when people need reassurance. It respects the clinician's role, keeps people in control of their information, and treats AI as a carefully bounded support tool - never a substitute for professional care.

Logo narrative

The preferred mark is a soft, open C that becomes a gentle pulse. The open form suggests an invitation to care, while the pulse represents both life and timely action. Its rounded geometry communicates warmth and accessibility; its precision reflects dependable health technology.

Use deep teal as the primary brand color for trust and steadiness. Reserve emergency red for critical calls to action only. This gives Curais a distinctive identity: calm by default, unmistakably urgent when it needs to be.

Curais Product Requirements Document

Product statement

Curais helps people act safely in a health moment, then keep the information needed for better follow-up care. SnapAid becomes the Immediate Care module inside Curais: fast, non-diagnostic first-aid guidance with direct emergency escalation.

Problem and audience

People in India, particularly in Tier 2/3 cities, often lack a clear next step during a health incident and arrive at a clinic without an accessible longitudinal record. The initial users are patients/caregivers needing immediate guidance and small clinics needing a simple patient-record workflow.

Product principles

- Safety before engagement: direct emergency escalation must remain visually dominant.
- Guidance, not diagnosis: never present an AI output as a confirmed condition.
- Consent and minimum data: collect only what is necessary for each capability.
- Mobile, low-bandwidth, and local-language first.

Releases

R1 — Curais Immediate Care (existing SnapAid, rebranded)

- Free-text symptom guidance and curated first-aid playbooks.
- Critical-vs-basic signs, emergency and poison-control calling.
- Safety disclaimer, local emergency-number detection, rate limiting, and feedback reporting.
- No account and no retained clinical history required.

R2 — Personal Care Record

- Account creation, verified mobile number, profile, allergies, medications, conditions, and emergency contacts.
- User-controlled incident history: save, export, or delete a SnapAid/Curais guidance session.
- Consent screen before any health information is saved.

R3 — Curais Clinic

- Clinic/staff workspace, patient lookup with explicit patient consent, visit notes, and structured vitals.
- Role-based access, audit trail, and patient sharing controls.
- Offline draft capture and safe conflict resolution; no silent overwrite.

R4 — Intelligence and connected care

- Clinician-reviewed decision support, device/vitals ingestion, care reminders, and aggregate, de-identified operational insights.
- Emergency workflows and public-health integrations only after legal, clinical, and security validation.

Success measures

Area Measure
Immediate care guidance completion rate, emergency CTA visibility/click rate, harmful-output reports
Trust consent completion, saved-record opt-in, deletion/export success
Clinic weekly active clinics, visit documentation completion, median record-retrieval time
Safety critical incidents, time to triage review, security/audit exceptions

Explicit non-goals for R1

- Diagnosis, prescriptions, or medication dosing generated by AI.
- Storing identifiable health data without an account and explicit consent.
- Autonomous emergency dispatch or clinical decision-making.

Curais Technical Architecture Document

Architecture direction

Keep the current Next.js application as the R1 presentation and immediate-guidance layer. Introduce a dedicated authenticated data platform only when R2 begins; do not retrofit personal health records into browser session storage or the current AI endpoint.

mermaid

flowchart LR

U[Patient / caregiver] --> W[Curais web or mobile client]

W --> G[API gateway]

G --> I[Immediate-care service]

I --> P[Curated playbooks]

I --> M[Guardrailed AI provider]

W --> A[Identity and consent service]

A --> R[Encrypted health-record service]

C[Clinic workspace] --> G

G --> L[Immutable audit log]

R --> L

R1 components

- app/page.js: symptom entry, quick options, safety CTA.
- app/api/route.js: server-side AI call, validation, and IP rate limit.
- data/quickAid.js: curated first-aid playbooks, preferred for recognized emergencies.
- context/ResultsContext.js: short-lived browser session guidance only.

R2+ components

- Managed identity provider with OTP/phone verification, session rotation, and MFA for clinical users.
- Relational transactional store for accounts, consent, clinic membership, and records; encrypted object storage for attachments.
- API boundary that authenticates every request, authorizes per resource, validates schemas, applies rate limits, and emits audit events.
- Background jobs for notifications and exports; no health data in analytics or error-monitoring payloads.

Data boundaries

Class Examples Handling
Public static first-aid content versioned, reviewed content repository
Sensitive symptom text, health history, vitals encrypted transit/storage, least privilege
Highly sensitive identity + health record, clinical notes separate access controls, audit every read/write/export

Key technical decisions

- Curated content wins for known high-risk scenarios; AI is a fallback with a strict schema and escalation policy.
- The model must receive the minimum symptom context; provider retention/training settings must be contractually reviewed before R2.
- Store an immutable content/version identifier with saved guidance so it can be explained or withdrawn later.
- Design APIs around patient-controlled sharing grants rather than clinic-wide record visibility.

Curais Security Assessment (Initial)

Scope and current posture

This assessment covers the current SnapAid codebase and the planned Curais expansion. SnapAid currently sends user-entered symptom text to a third-party AI API, keeps results in browser session storage, and provides emergency guidance. It does not currently implement accounts or persistent patient records.

Current strengths

- AI key is server-side, not shipped to the browser.
- Input length is limited and the guidance response is schema-normalized.
- API requests have in-memory IP rate limiting.
- Curated emergency paths avoid an AI request.

Priority findings

Priority	Finding	Required action
P0	AI content can be unsafe or misleading despite structured output.	Add clinical-review governance, high-risk keyword routing to curated emergency content, output safety checks, and user reporting before a public Curais launch.
P0	Health information is transmitted to a third-party model provider.	Publish a clear privacy notice; minimize/strip identifiers; verify provider data-retention, regional processing, and no-training terms; obtain explicit consent before record saving.
P1	Current in-memory rate limiter does not work reliably across serverless instances.	Move to a shared, durable rate-limit store with bot/abuse monitoring.
P1	No authentication/authorization model exists for future records.	Implement scoped RBAC/ABAC, patient sharing grants, MFA for clinicians, and audit logs before storing PHI/health records.
P1	No formal security headers/CSP are visible in the current app configuration.	Add CSP, HSTS, frame protections, secure cookies, dependency scanning, and production error redaction.
P2	sessionStorage keeps guidance on the device until the browser session ends.	Explain this behavior; use encrypted server storage only after opt-in and provide delete/export controls.

Required controls before R2

- Threat model and data-flow review for every health-data feature.
- Encryption in transit and at rest; managed keys and rotation policy.

- Separate production environments, secret manager, least-privilege service roles, and access review.
- Tamper-evident audit events for record access, changes, shares, exports, and admin actions.
- Incident response playbook, backups/restore test, vulnerability scanning, and penetration test.
- Legal/privacy review for Indian health-data obligations and every target market. This document is not legal advice.

Curais Feature Ticket List

P0 — Immediate Care safety and rebrand

ID	Ticket	Acceptance criteria
CUR-001	Rebrand SnapAid to “Curais Immediate Care”	Curais identity used consistently; existing guidance flow stays functional.
CUR-002	Add emergency intent router	High-risk intents show curated emergency action before any model output.
CUR-003	Add AI safety evaluation set	Clinician-reviewed test cases cover chest pain, stroke signs, choking, anaphylaxis, poisoning, burns, pregnancy, and child scenarios.
CUR-004	Replace serverless rate limiter	Limit is shared across instances and returns consistent retry information.
CUR-005	Add feedback/reporting	User can flag unsafe or unhelpful guidance without submitting unnecessary personal data.
CUR-006	Privacy and consent copy	Explains AI processing, no-diagnosis boundary, and data retention in plain language.

P1 — Personal Care Record

ID	Ticket	Acceptance criteria
CUR-101	Account and phone verification	Authenticated users have secure sessions; sign-out invalidates session.
CUR-102	Health profile	User can add/edit/delete allergies, medications, and emergency contacts.
CUR-103	Save guidance with consent	No guidance is persisted until the user explicitly chooses Save.
CUR-104	Export and delete data	User can export their records and request/account deletion from settings.
CUR-105	Consent ledger	Every sharing/saving consent is versioned, timestamped, and revocable.

P2 — Clinic workspace

ID	Ticket	Acceptance criteria
CUR-201	Clinic onboarding and roles	Owner, clinician, assistant, and support roles have least-privilege access.
CUR-202	Patient sharing grant	Clinic sees a record only after patient authorization or documented lawful workflow.
CUR-203	Visit record and vitals	Changes are attributed, timestamped, and auditable.
CUR-204	Offline drafts	Local drafts are encrypted where supported and require review on sync.
CUR-205	Access audit view	Patient/admin can view meaningful access events.

Curais Frontend Specification

Information architecture

R1 has one immediate-care experience. R2 introduces a patient home with Immediate Care, My Health, and Care Team. Clinic users access a distinct, clearly labelled workspace; never mix clinical controls into the emergency flow.

Immediate Care flow

1. Home: Curais wordmark, “Get help now” symptom entry, common emergency buttons, persistent emergency CTA.
2. Input: voice/text support, clear data-use notice, character limit, and loading state.
3. Result: emergency CTA first; possible concern and concise action line; numbered steps; red flags; disclaimer; “save to My Health” only for signed-in users who opt in.
4. Error/offline: say what is unavailable, retain curated quick-aid options and emergency CTA.

Visual direction

- Brand personality: calm, capable, human, never alarmist except when escalation is needed.
- Core palette: deep teal/ink for trust, soft mint/ivory backgrounds, emergency red reserved solely for urgent actions.
- Typography: rounded, high-legibility sans for headings and a neutral text face; support Devanagari and other target scripts.
- Logo use: compact symbol + wordmark; the emergency module may use “Curais / Immediate Care,” not a separate competing master brand.

Accessibility and interaction requirements

- Meet WCAG 2.2 AA: contrast, keyboard navigation, visible focus, semantic headings, labelled controls, and status announcements.
- Touch targets minimum 44 by 44 CSS pixels.
- Never rely on color alone for critical/basic status.

- Keep emergency CTA available on every mobile result screen without obscuring content.
- Reduce animation when prefers-reduced-motion is enabled.

Responsive behavior

- Design mobile first from 320px width; one-column urgent flow.
- Tablet/desktop may widen reading cards but preserve the first action and emergency CTA above the fold.
- Treat slow/unstable networks as normal: low-weight assets, skeleton/loading states, no blocking animation.

Key UI states

State Required behavior
Emergency intent show direct action and curated guidance immediately
AI loading disable duplicate submit, keep emergency call accessible
Invalid/unsafe result show retry plus emergency/safety path; do not render partial guidance
Signed out allow immediate care; explain sign-in benefit before saving
Consent required concise purpose, data categories, retention, revoke link